

Siya Health®

We didn't build a clinic. We built the layer of care that was always missing — before, during, and after the visit.

Physician-led telehealth · care that follows through, before and after the visit · multi-state operations

The problem

**The doctor makes the right care plan.
The patient doesn't always get to live it.**

Care breaks down across three phases — before the visit, during it, and after. Not because of missing technology. Because of missing follow-through.

Pre-visit

At the visit

Post-visit

Pre-visit

Before the visit, the wrong patients get through — and the right ones don't show up.

- Poor eligibility review → mismatched expectations, refunds, chargebacks, bad reviews

- No personalized reminders → no-shows

Fix

Our concierge team reviews eligibility before booking and calls patients personally to confirm.

At the visit

During the visit, the doctor is doing admin work instead of practicing medicine.

- Incomplete intake → doctor manually gathers history

- No documentation/order support → less patient-facing time

Fix

Our concierge team pre-builds the intake summary and scribes live, so the doctor's time is spent only on the patient.

Post-visit

After the visit, care quietly falls apart.

- Pharmacy coordination failures → prescriptions don't reach the patient
- No structured follow-up → low compliance, interrupted care

Fix

Our concierge team owns pharmacy coordination and structured follow-up — checking in, explaining what to expect, catching problems before they interrupt care.

Today, we've solved this with trained people operating disciplined protocols on top of existing tools. As those protocols mature, they become the blueprint for technology we build ourselves — proven first, encoded second.

Traction

Revenue growing steadily since February.
Cash-flow positive.

2,700+

Patients treated (growing)

1,200+

Neurocognitive evaluations

4 states

Multi-state operations

Cash-flow+

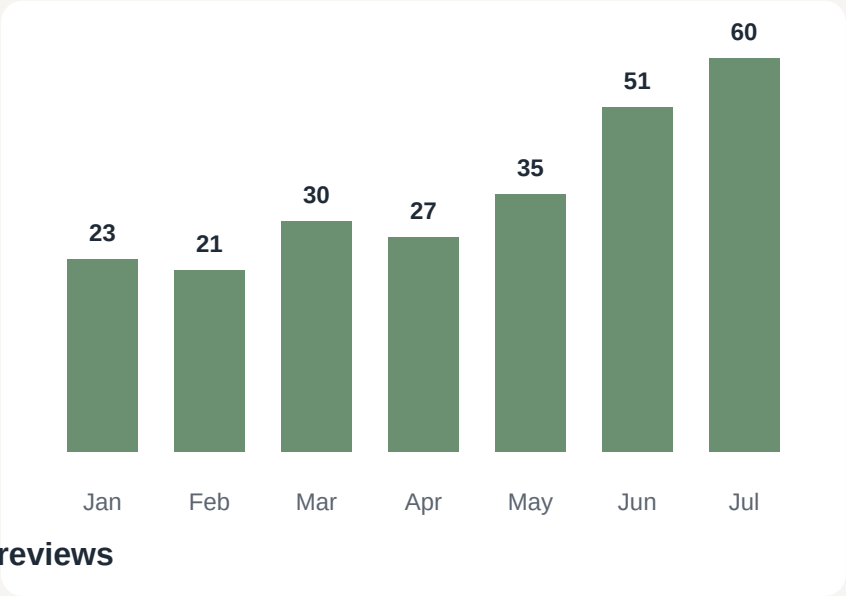
Operations

Reviews: 4.9★ on Google (100+ reviews) · 4.75★ average across 600+ third-party provider reviews

No-shows: Declining trend Q4 2025–Q1/Q2 2026 as trained MAs and automated reminders took effect — now plateaued, pointing to factors beyond reminders we're actively addressing next.

[Retention: 3-month cohort data pending final team calculation — leave placeholder, do not fabricate]

Monthly gross revenue — 2026



Where we are, honestly

We saw the risk before you did — and we're already fixing it.

Proof

Founder-led relationships filled every provider's panel, not just the founder's own. It worked — 2,700+ patients prove it.

Risk

One person's time doesn't scale. We knew that going in.

Fix

September 2026 — a fractional COO joined, already driving fundraising readiness and employer/B2B pipeline. This round adds the clinical capacity to match.

Business model

Cash-pay, recurring, no insurance risk.

\$149 Initial evaluation One-time, self-pay	\$79 / mo Ongoing care — non-controlled Recurring membership	\$149 / mo Ongoing care — controlled Recurring membership
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Siya Health Inc (MSO) + affiliated professional corporation(s) — standard, legally reviewed structure. B2B/employer pricing and MSO management-fee terms are being finalized as the company adds affiliated providers and employer partnerships.

150+ recurring members and growing. Cash-pay means no payer contracts, no reimbursement risk, revenue from day one.

Market — bottom-up

21.7M working adults live with a diagnosable condition affecting attention, sleep, or focus — most of them undiagnosed or untreated.

Layer	Definition	Size
TAM	US clinical services for adult ADHD + clinical insomnia	~\$18B
SAM	Employed adults with either condition, telehealth-willing, cash-pay comparable	~21.7M people
SOM	Year-3, capacity-derived — 8–10 FTEs at ~800 patients each	~\$3.8M–4.8M

Built bottom-up: 163M employed U.S. adults (BLS 2026) · 15.5M diagnosed adult ADHD, 63% employed · ~10% clinical insomnia disorder among employed adults · overlap-adjusted (44% ADHD/insomnia comorbidity, per published clinical data). Existing independent contractor network (8 providers) represents additional capacity not included in this Year-3 estimate. Full sizing methodology available in diligence.

Competition

Different job. Not a feature checklist.

Employer mental-health platforms (e.g. Lyra Health)	Broad coverage, not built for objective, condition-specific clinical depth
Generic access & counseling (EAPs, episodic telehealth)	Gets you seen once. No structured monitoring, no concierge follow-through
DTC brain-training / wellness apps	Games and self-assessment. Not clinical, no escalation if something real is found
Siya	Clinical-grade screening + a trained concierge team + real escalation to licensed providers, on your schedule, not office hours

No feature matrix. No 'everyone top-right' quadrant.

Go-to-market

Working channels today. Capacity is the job this round funds.

01 · Today

Channels that fill panels

Existing channels + founder-led fill — ~\$80 CAC estimate (current channel estimate, not yet a mature/audited figure).

02 · Already in motion

Fractional COO

Fractional COO since Sept 2026 — driving fundraising readiness and B2B pipeline, self-funded.

03 · This round's job

Clinical capacity

Add clinical capacity to serve proven demand.

04 · Timeline

Vendor-delivered CAC

Professional-vendor-driven CAC expected within ~6 months of engaging a partner — no calendar promise.

Why now

Demand is already here. Healthcare hasn't caught up to what working professionals actually need.

While carefully screening ADHD patients, we noticed something worth building on: a negative ADHD or PTSD screen doesn't mean nothing is there. Healthcare screens for cognitive and mental health reactively — a PHQ flag in Epic, dementia screening starting at 65. We think tracking these markers longitudinally, especially for working professionals under sustained pressure, is worth building now.

Discipline: SOPs, QA, and multi-state compliance already in place — the bottleneck is capacity and founder bandwidth, not the care model.

Use of funds

We're raising \$500k to hold capacity and hand off acquisition.

01 Clinical capacity & retention · ~\$300k

Two PA/NP hires (licensing, DEA, malpractice, benefits) + medical director multi-state supervision.

02 Leadership capacity · ~\$100k

Sustains the fractional COO retainer through the 18–24 month runway, already in motion since September 2026.

03 Professional marketing partnership · ~\$100k

Healthcare-specific vendor once capacity is stable.

Objective: Stabilize capacity against proven demand, free founder bandwidth, then replace founder-led acquisition with a professional partner.

Founders

Physicians who built the practice before asking for capital.



Dr. Sneh Pandey

Founder · CEO

- Board-certified physician
- 10,000+ patient encounters at Siya Health across primary care, obesity, hospital medicine, and ADHD care
- Built multi-state practice to 2,700+ patients, cash-flow positive, bootstrapped



Dr. Swati Pandey

Co-founder

- Board-certified physician, licensed in Pennsylvania
- Co-founder and clinical advisor — shapes clinical protocol standards across Siya Health

I was strict about controlled substances in my own training, long before Siya existed. I'd read about companies getting sued for overprescribing, and I didn't want us to become another cautionary tale. So the pill counts, the random drug screens, the compliance formalities — none of that came from a regulation checklist. It came from not wanting my name attached to a company that cut corners on this.

— Dr. Sneh Pandey

**Years spent learning how to deliver better care.
This round scales what already works.**

Seed round

\$500,000

Clinical capacity & retention · Leadership capacity · Professional marketing partnership

Siya Health®

Let's talk.

Dr. Sneh Pandey · Founder & CEO

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